

100-0-3

Carcinoma, urothelial 8120/3

Site: bladder, trigone C67.0

hw 4/7/11

PATIENT HISTORY:

Bladder cancer.

PRE-OP DIAGNOSIS: Bladder cancer.

POST-OP DIAGNOSIS: Same.

PROCEDURE: Cystectomy.

FINAL DIAGNOSIS:

PART 1: URETERAL MARGIN, RIGHT, RADICAL CYSTECTOMY -

- A. BENIGN URETER.
- B. NO EVIDENCE OF IN-SITU OR INVASIVE CARCINOMA.

PART 2: URETERAL MARGIN, LEFT, RADICAL CYSTECTOMY -

- A. FOCAL UROTHELIAL ATYPIA (See comment).
- B. NO EVIDENCE OF IN-SITU OR INVASIVE CARCINOMA.

PART 3: BLADDER, URETHRA, AND VAGINAL CUFF, RADICAL CYSTECTOMY -

- A. UROTHELIAL CARCINOMA, INVASIVE, WHOISUP HIGH GRADE, WITH MICROPAPILLARY FEATURES.
- B. CARCINOMA IS AT LEAST 5.0 CM IN GREATEST DIMENSION.
- C. TUMOR INVADES THROUGH THE DETRUSOR MUSCLE AND EXTENSIVELY INVOLVES PERIVESICULAR ADIPOSE TISSUE.
- D. CARCINOMA IS PRESENT AT THE PERIVESICAL ADIPOSE TISSUE (ADVENTITIAL) RESECTION MARGIN AT THE POSTERIOR ASPECT AND DOME OF BLADDER (Blocks 3I, 3J AND 3P).
- E. FOCAL CARCINOMA IN-SITU OF BOTH URETERS IS PRESENT; URETERIC RESECTION MARGINS ARE FREE OF IN-SITU OR INVASIVE MALIGNANCY.
- F. THE URETHRAL MARGIN IS FREE OF IN-SITU OR INVASIVE MALIGNANCY.
- G. EXTENSIVE ANGIOLYMPHATIC INVASION IS IDENTIFIED.
- H. FOCAL PERINEURAL INVASION IS IDENTIFIED.
- I. THE VAGINAL WALL IS FREE OF TUMOR.
- J. PATHOLOGIC STAGE: pTNM = pT3b N1 Mx.
- K. INCIDENTAL BENIGN LEIOMYOMA (0.8 CM.) IN PERIVESICLE ADIPOSE TISSUE.

PART 4: LYMPH NODES, RIGHT PELVIC, EXCISION -

METASTATIC UROTHELIAL CARCINOMA IN ONE OF TWO LYMPH NODES EXAMINED (1/2).

PART 5: LYMPH NODES, LEFT PELVIC, EXCISION -

METASTATIC UROTHELIAL CARCINOMA IN SIX OF SEVEN LYMPH NODES EXAMINED (6/7).

COMMENT:

Part 2: The permanent section of the left ureteral resection margin shows mild urothelial atypia, which, at most, represents low grade intraurothelial neoplasia (dysplasia). There is no evidence of carcinoma in situ or invasive carcinoma at the margin.

Part 3: A section of the bladder wall with adjacent vaginal wall (block 3C) shows tumor infiltrating between smooth muscle bundles, which are felt to represent the outer bundles of detrusor muscle, rather than vaginal wall smooth muscle. Vaginal wall involvement by tumor is therefore not evident.

MICROSCOPIC:

SYNOPTIC DATA - PRIMARY URINARY BLADDER TUMORS

SPECIMEN TYPE:

Radical cystectomy

TUMOR SITE:

Trigone, Left lateral wall, Posterior wall, Dome

TUMOR SIZE:

Greatest dimension: 5.0 cm

Additional dimensions: 4.5 and 1.4 cm

HISTOLOGIC TYPE:

Urothelial (transitional cell) carcinoma with variant histology: micropapillary

ASSOCIATED EPITHELIAL LESIONS:

None identified

HISTOLOGIC GRADE:

Urothelial carcinoma - High-grade

TUMOR CONFIGURATION:

Flat, Ulcerated

PATHOLOGIC STAGING (pTNM):

pT3b

pN1

Number of nodes examined: 9

pMx

MARGINS:

Margin(s) involved by invasive carcinoma

Margin(s): Adventitial/ perivesicular soft tissue

VENOUS/LYMPHATIC (LARGE/SMALL) VESSEL INVASION (V/L):

Present

DIRECT EXTENSION OF INVASIVE TUMOR:

Perivesical fat

UUID: F974D857-0AA3-4BAA-9C65-A71F27A72877
TCGA-BT-A20R-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Notes		☒
Case is (Link's):	QUALIFIED	DISQUALIFIED
Reviewer Initials	hw	4/7/11